Droperidol

Mechanism of action

- Droperidol blocks dopamine and alpha receptors centrally, resulting in sedation, reduced agitation and a state of mental detachment, and antiemetic action.

Authorised practice levels

- Paramedics, ICPs and CCPs.

Indications

- ✓ Patients aged greater than or equal to 12 years with acute behavioural disturbance causing a mild to moderate risk to safety, when olanzapine has not been administered or is ineffective.
- ✓ Severe pain, usually in conjunction with an opiate, associated with one of the following:
  - Chronic use of opiates.
  - Severe psychological distress.
  - Nausea and/or vomiting.
- ✓ Management of nausea and agitation or pain that does not respond to an opiate during the last days of life.
- ✓ Nausea and/or vomiting:
  - That persists despite ondansetron, or
  - Where motion sickness is a component of the patient's nausea and/or vomiting, or
  - The patient has cannabinoid hyperemesis syndrome.
- ✓ Vertigo.
- ✓ Primary headache that is severe.

Contraindications

- ✗ Known severe allergy.
- ✗ Aged less than 12 years.
- ✗ Pregnancy (unless indicated for acute behavioural disturbance).

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Cautions

- Altered level of consciousness. This will increase and prolong the effects.
- Parkinson's disease. There is a risk of worsening the movement disorder associated with Parkinson's disease.
- Concurrent administration of ketamine or midazolam. This will increase and prolong the effects.
- Aged greater than or equal to 75 years, particularly if frail. The effects of droperidol will be increased and prolonged in this cohort.
- Signs of shock. Droperidol may make shock worse.

Use in pregnancy or when breastfeeding

- The likelihood of a pregnant or breastfeeding patient requiring droperidol for acute behavioural disturbance is very low, however the balance of risk is such that it should be administered if indicated in this context.
- For all other indications, the balance of risk is not in favour of administration of droperidol and therefore it is contraindicated.

Dosage

- For analgesia:
  - 0.625 mg IV/IM for an adult, once only.
- For acute behavioural disturbance:
  - 10 mg IV/IM for an adult.
  - Consider reducing the dose to 5 mg if a caution is present.
  - The dose may be repeated once after 20 minutes.
- For care during the last days of life:
  - 1.25 mg IV or 2.5 mg IM/SC.
  - The dose may be repeated as required.
- For nausea and/or vomiting:
  - 0.625 mg IV/IM for an adult, once only.
- For vertigo:

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- 1.25mg IV/IM for an adult, once only.
- Consider reducing the dose to 0.625 mg IV/IM if a caution is present.

- For primary headache that is severe:
  - 1.25 mg IV/IM for an adult, once only.
  - Consider reducing the dose to 0.625 mg IV/IM if a caution is present.

Administration

- IV administration in acute behavioural disturbance:
  - Draw up the required dose and dilute to a total of 10 ml using 0.9% sodium chloride.
  - Administer IV over 1-2 minutes.

- IV administration for nausea and/or vomiting, an adjunct to analgesia, vertigo or primary headache that is severe:
  - Draw up 2.5 mg and dilute to a total of 4 ml using 0.9% sodium chloride. This gives a 0.625 mg/ml solution.
  - Administer the required dose as an IV bolus.

- IM administration: Administer undiluted. The preferred IM site is the lateral thigh. If this site is not suitable use the lateral upper arm.

- SC administration:
  - Administer undiluted subcutaneously into the abdominal wall.
  - There is no need to sterilise the skin at the site of injection unless the skin is visibly contaminated.

Common adverse effects

- Hypotension. This may occur if a large IV dose is administered rapidly.

Usual onset of effect

- IV/IM: 5-10 minutes.

Usual duration of effect

- 4-6 hours.

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Usual preparation

- Ampoule containing 2.5 mg in 1 ml.

Pharmacokinetics

- Droperidol is predominantly metabolised in the liver with metabolites being excreted in the urine.
- There are no significant effects from liver impairment on acute administration.

Common interactions

- Intoxication. Droperidol will have increased sedative effects if the patient is intoxicated with alcohol or has taken recreational drugs.
- Sedative drugs. Concurrent administration with other sedative drugs (such as olanzapine or midazolam) will result in an increased sedative effect.

Additional information

- Droperidol has been reported to prolong the QT interval. This generally involved repeated and/or high doses and one or two doses are safe, even if the patient is known to have a prolonged QT interval.
- Droperidol may cause dyskinesia (abnormal, uncoordinated and involuntary movements) but this is unusual following one or two doses.

More Information

Version history

16 June 2026 1.1.0.1
Update: Indications and dosage updated.

9 June 2025 1.0.5.3
Update: Indications and contraindication updated.

11 September 2023 1.0.4
Update: Indications and dosage updated.

22 November 2022 1.0.1
Update: Aligned with analgesia CPG.

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